Additional Subgroup Analyses and Supplementary Materials

Diagnostic Thresholds of Fever and Hypothermia for Predicting Mortality and Sepsis in Young Infants: A Systematic Review and Meta-analysis

Contents **Page Number**

eTable 1. PRISMA Checklist

Section and Topic	Item #	Checklist item	Location where item is reported
TITLE			
Title	1	Identify the report as a systematic review.	Title, 1
ABSTRACT			
Abstract	2	See the PRISMA 2020 for Abstracts checklist.	2-3
INTRODUCTION			
Rationale	3	Describe the rationale for the review in the context of existing knowledge.	4
Objectives	4	Provide an explicit statement of the objective(s) or question(s) the review addresses.	5
METHODS			
Eligibility criteria	5	Specify the inclusion and exclusion criteria for the review and how studies were grouped for the syntheses.	5-6
Information sources	6	Specify all databases, registers, websites, organisations, reference lists and other sources searched or consulted to identify studies. Specify the date when each source was last searched or consulted.	6
Search strategy	7	Present the full search strategies for all databases, registers and websites, including any filters and limits used.	eTable 2a, eTable 2b
Selection process	8	Specify the methods used to decide whether a study met the inclusion criteria of the review, including how many reviewers screened each record and each report retrieved, whether they worked independently, and if applicable, details of automation tools used in the process.	6-7
Data collection process	9	Specify the methods used to collect data from reports, including how many reviewers collected data from each report, whether they worked independently, any processes for obtaining or confirming data from study investigators, and if applicable, details of automation tools used in the process.	7
Data items	10a	List and define all outcomes for which data were sought. Specify whether all results that were compatible with each outcome domain in each study were sought (e.g. for all measures, time points, analyses), and if not, the methods used to decide which results to collect.	5-6
	10b	List and define all other variables for which data were sought (e.g. participant and intervention characteristics, funding sources). Describe any assumptions made about any missing or unclear information.	6-7

Study risk of bias assessment	11	Specify the methods used to assess risk of bias in the included studies, including details of the tool(s) used, how many reviewers assessed each study and whether they worked independently, and if applicable, details of automation tools used in the process.	7
Effect measures	12	Specify for each outcome the effect measure(s) (e.g. risk ratio, mean difference) used in the synthesis or presentation of results.	6
Synthesis methods	13a	Describe the processes used to decide which studies were eligible for each synthesis (e.g. tabulating the study intervention characteristics and comparing against the planned groups for each synthesis (item #5)).	8
	13b	Describe any methods required to prepare the data for presentation or synthesis, such as handling of missing summary statistics, or data conversions.	8
	13c	Describe any methods used to tabulate or visually display results of individual studies and syntheses.	8
	13d	Describe any methods used to synthesize results and provide a rationale for the choice(s). If meta-analysis was performed, describe the model(s), method(s) to identify the presence and extent of statistical heterogeneity, and software package(s) used.	8
	13e	Describe any methods used to explore possible causes of heterogeneity among study results (e.g. subgroup analysis, meta-regression).	8
	13f	Describe any sensitivity analyses conducted to assess robustness of the synthesized results.	n/a
Reporting bias assessment	14	Describe any methods used to assess risk of bias due to missing results in a synthesis (arising from reporting biases).	7
Certainty assessment	15	Describe any methods used to assess certainty (or confidence) in the body of evidence for an outcome.	n/a
RESULTS			
Study selection	16a	Describe the results of the search and selection process, from the number of records identified in the search to the number of studies included in the review, ideally using a flow diagram.	8
	16b	Cite studies that might appear to meet the inclusion criteria, but which were excluded, and explain why they were excluded.	8, eTable 3, eTable 4
Study characteristics	17	Cite each included study and present its characteristics.	9, eTable 5
Risk of bias in studies	18	Present assessments of risk of bias for each included study.	9, eFigures 2-5
Results of individual studies	19	For all outcomes, present, for each study: (a) summary statistics for each group (where appropriate) and (b) an effect estimate and its precision (e.g. confidence/credible interval), ideally using structured tables or plots.	Table 1, Table 2

Results of syntheses	20a	For each synthesis, briefly summarise the characteristics and risk of bias among contributing studies.	9
	20b	Present results of all statistical syntheses conducted. If meta-analysis was done, present for each the summary estimate and its precision (e.g. confidence/credible interval) and measures of statistical heterogeneity. If comparing groups, describe the direction of the effect.	10-13, Figures 2a-2c
	20c	Present results of all investigations of possible causes of heterogeneity among study results.	10-13
	20d	Present results of all sensitivity analyses conducted to assess the robustness of the synthesized results.	n/a
Reporting biases	21	Present assessments of risk of bias due to missing results (arising from reporting biases) for each synthesis assessed.	Not done due to small number of studies per study question
Certainty of evidence	22	Present assessments of certainty (or confidence) in the body of evidence for each outcome assessed.	n/a
DISCUSSION			
Discussion	23a	Provide a general interpretation of the results in the context of other evidence.	14-15
	23b	Discuss any limitations of the evidence included in the review.	15-16
	23c	Discuss any limitations of the review processes used.	16
	23d	Discuss implications of the results for practice, policy, and future research.	16
OTHER INFORMATION			
Registration and protocol	24a	Provide registration information for the review, including register name and registration number, or state that the review was not registered.	Title page, 5
	24b	Indicate where the review protocol can be accessed, or state that a protocol was not prepared.	Title page, 5
	24c	Describe and explain any amendments to information provided at registration or in the protocol.	n/a
Support	25	Describe sources of financial or non-financial support for the review, and the role of the funders or sponsors in the review.	Title page
Competing interests	26	Declare any competing interests of review authors.	Title page

Availability of data, code and other materials	27	Report which of the following are publicly available and where they can be found: template data collection forms; **data extracted from included studies**; data used for all analyses; analytic code; any other materials used in the review.	Main paper, 23

From: Page MJ, McKenzie JE, Bossuyt PM, Boutron I, Hoffmann TC, Mulrow CD, et al. The PRISMA 2020 statement: an updated guideline for reporting systematic reviews. BMJ 2021;372:n71. doi: 10.1136/bmj.n71

eFigure 1. Rectal vs. Axillary Temperature Conversion Chart

Rectal		Axillary	
°F	°C	°C	°F
95	35	34.5	94.1
95.9	35.5	35	95
96.8	36	35.5	95.9
97.7	36.5	36	96.8
98.6	37	36.5	97.7
99.5	37.5	37	98.6
100.4	38	37.5	99.5
101.3	38.5	38	100.4
102.2	39	38.5	101.3
103.1	39.5	39	102.2
104	40	39.5	103.1
		40	104

eTable 2. Characteristics of additional studies identified reporting Hazard Ratios (HR) only

The following studies report on individual clinical signs from our study protocol, but were excluded due to not reporting an outcome measure of interest from our protocol (OR, RR, sensitivity or specificity)

Author and year	Study title	Study design and dates	Study setting	Patient population & selection	Age (days)	Sample N	Sign(s) from current study's protocol reported in paper	Description of clinical sign	Clinical sign test cadre	Reference standard description	Reference cadre

Darmstadt, GL. *EClinical Medicine.* 2025	Association of clinical signs of possible serious bacterial infections identified by community health workers with mortality of young infants in South Asia: a prospective, observational cohort study	Prospective, population-based cohort study November 1, 2011- December 31, 2013	Community-based home visits by trained CHWs in five representative sites in Bangladesh, India, Pakistan	All live-born infants enrolled at study sites and followed prospectively through household visits by CHWs	0–59 d	63017	-Hypothermia -Fever	Temperature: <35.5°C (hypothermia), >37.5°C (fever)	CHWs via home visits	Possible Serious Bacterial Infection: clinical assessment and verbal autopsy for cause of death assignment; mortality risk modeling via Cox regression	Physicians
Huang, X. *Transl Pediatr.* 2023	A nomogram for predicting death for infants born at a gestational age of <28 weeks: a population-based analysis in 18 neonatal intensive care units in northern China	Retrospective, large-scale, multicenter clinical study January 1, 2015- December 31, 2018	18 hospitals with Level III NICUs (17 tertiary and 1 private comprehensive hospital) in China (Northern region)	Extremely preterm infants (GA <28 weeks) receiving neonatal resuscitation and transferred to NICU	0 d- NICU discharge	568 cases	-Hypothermia	Axillary temperature <36 °C within 1 h after admission	NICU clinicians and neonatal care team	In-hospital mortality: death during NICU admission	Hospital NICU records and clinical documentation
Mengesha, HG. *BMC Pregnancy Childbirth*. 2016	Survival of neonates and predictors of their mortality in Tigray region, Northern Ethiopia: prospective cohort study	Multivariate Cox-proportional hazard model analysis of a prospective cohort study April-July 2014	7 randomly selected public hospitals in Tigray region, Northern Ethiopia	Inborn neonates (excluding home deliveries and neonates arriving >6 h after birth)	0-28 d	1152	-Hypothermia	Body temperature <36.5 °C	Skilled birth attendants and health workers in hospital setting	Neonatal death: status confirmed during hospital stay and follow-up	Hospital clinical staff and data collectors

bpm= Breaths per minute, CHW= community health worker, d= Days, GA= Gestational age, NICU= Neonatal Intensive Care Units, NR=Not reported

Figure 2. Newcastle-Ottawa Scale for Cohort Studies

Study	Selection				Total score for selection	Outcome				Total score for outcome	Overall Quality Score	Overall Quality Ranking
	Representativeness of the exposed cohort	Selection of the non exposed cohort	Ascertainment of exposure	Appropriateness of interval between index tests and reference standard*		Assessment of outcome	Appropriateness of reference standard**	Adequacy of follow-up length	Adequacy of follow up of cohorts			
Ashrafzadeh 2019	-	★	★	★	3	★	★	-	-	2	5	Good
Bang 2005a	★	★	★	★	4	★	★	★	★	4	8	Good
Bang 2005b	★	★	★	★	4	★	★	★	★	4	8	Good
Bekhof 2013	-	★	★	★	3	★	★	★	★	4	7	Good
Darmstadt 2011	★	★	★	★	4	★	★	★	★	4	8	Good
Desalew 2020	-	★	★	★	3	★	★	-	-	2	5	Good
Duke 2005	★	★	★	★	4	★	★	-	★	3	7	Good
English 2004	-	★	★	-	2	★	★	★	-	3	5	Fair
Hailemeskel 2022	-	★	★	★	3	★	★	-	★	3	6	Good
John 2015	-	★	★	-	2	★	★	★	★	4	6	Good
Lee 2019	-	★	★	★	3	★	★	★	★	4	7	Good
Mathur 2007	-	★	★	★	3	★	★	-	★	3	6	Good
Orfanos 2022	★	★	★	-	3	★	★	★	★	4	7	Good

Padula 2014	-	★	★	★	3	★	★	★	★	4	7	Good
Ramgopal 2019b	-	★	★	★	3	★	★	★	★	4	7	Good
Ronfani 1999	-	★	★	-	2	-	★	★	★	3	5	Fair
Thapa 2013	-	★	★	-	2	★	★	★	★	4	6	Fair
Verstraete 2018	-	★	★	★	3	★	★	★	★	4	7	Good
Weber 2003	★	★	★	-	3	★	★	★	★	4	7	Good
YICSG 2008	★	★	★	★	4	★	-	★	★	3	7	Good
Zhang 2023a	-	★	★	★	3	★	★	-	★	3	6	Good

*We removed Selection question 4, "Demonstration that outcome of interest was not present at start of study," as our research population includes most infants presenting with a sign(s) of illness. We altered this question to reflect flow and timing appropriate to our review question: Was there an appropriate interval between index tests and reference standard?
**We added the following question to the outcome section (Outcome question 2): Was reference standard likely to correctly classify the target condition?

eTable 3. Adaptation and interpretation of Newcastle-Ottawa Scale for Cohort Studies

Thresholds for converting the Newcastle-Ottawa scales to Agency for Healthcare Research and Quality standards (good, fair, and poor):
Good quality: 3 or 4 stars in selection domain AND 1 or 2 stars in comparability domain* AND 2 or 3 stars in outcome/exposure domain
Fair quality: 2 stars in selection domain AND 1 or 2 stars in comparability domain* AND 2 or 3 stars in outcome/exposure domain
Poor quality: 0 or 1 star in selection domain OR 0 stars in comparability domain* OR 0 or 1 stars in outcome/exposure domain

*As we removed the comparability domain for this study, we removed this summary grading criteria

Adapted Newcastle Ottawa Quality Assessment Scale for Cohort Studies:
Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories.

Selection
1) Representativeness of the cohort
- a. Participants presented to health centers and hospitals or were recruited in the community - 1 point
- b. Participants were admitted to hospital/in specialized units/NICUs or excluded for being too ill to participate - 0 points

c. No description of the derivation of the cohort - 0 points

2) Selection of the non exposed cohort

a. Drawn from the same community as the exposed cohort - 1 point

b. Drawn from a different source - 0 points

c. No description of the derivation of the non exposed cohort - 0 points

3) Ascertainment of exposure

a. Secure medical records - 1 point

b. Structured interview- 1 point - study clinician measured signs or administered via interview using a structured questionnaire/form to parent/caregiver

c. Written self report - 0 points - index test was ascertained without an interview (form, no discussion with study clinician)

d. No description - 0 points

4) Was there an appropriate interval between index tests and reference standard? (sepsis outcomes: assumed simultaneous if not reported, death outcome: mark as appropriate if clearly described)

a. Appropriate= <4 hours - 1 point

b. Medium 4-12 hours - 0 points

c. High >12 hours - 0 points

d. No description - 0 points

<u>Outcome</u>

1) Assessment of outcome

a. Independent blind assessment - 1 point

b. Record linkage (culture/lab-confirmed infection or death) - 1 point

c. Unblinded assessment - 0 points

d. No description - 0 points

2) Was reference standard likely to correctly classify the target condition

a. Culture-confirmed infection - 1 point

b. Clinical sepsis diagnosed with access to laboratory investigations - 1 point

c. Mortality – 1 point

d. No description or laboratory investigations only available to a selective subgroup - 0 points

3) Was follow up long enough for outcomes to occur

a. Yes (participants were followed up for the outcome of interest until the end of their cohort age for death outcome and for at least 2 days for sepsis outcome)- 1 point

b. No (participants were followed up for less than the end of their cohort age or death outcome or less than 2 days for sepsis outcome. [For death outcome only: outcome was in-hospital mortality]) - 0 points

4) Adequacy of follow up cohorts

a. Complete follow-up - all subjects accounted for - 1 point

b. Small number of subjects lost to follow-up (<=20% LTF) or description provided of those lost- 1 point c. Large number of subjects lost to follow-up (>20% LTF) and no description of those lost - 0 points d. No statement on follow up - 0 points
Questions were tailored to reflect the research question of this study: 1. We removed the Comparability question, "Comparability of cohorts on the basis of the design or analysis" option a) study controls for ______________ (select the most important factor) Option b) study controls for any additional factor (This criteria could be modified to indicate specific control for a second important factor.) Our review question is designed to include low- and middle- income settings in which it is often infeasible to obtain sufficient information to control for confounding factors, such as birthweight and gestational age. We do not penalize for this expected characteristic of our included studies.
The NOS scale was applied using the cohort template for observational designs (prospective or retrospective) and the case-control template only when cases were selectively sampled (e.g., matched). For cohort studies, we adapted the NOS by: (1) replacing the item on outcomes not present at baseline with "Was there an appropriate interval between index tests and reference standard?"; (2) removing the comparability item, given the limited feasibility of adjusting for factors such as birthweight and gestational age in low-resource settings; and (3) adding "Was the reference standard likely to correctly classify the target condition?" For case-control studies, we (1) removed the Comparability section, consistent with our cohort adaptations, and (2) modified the exposure item, as chart review was common and considered acceptable despite the original scale's downgrade for medical record use.

eFigure 3. Newcastle-Ottawa Scale for Case Control Studies

Study	Selection				Exposure			Quality Score	Overall Quality
	Adequacy of the case definition	**Representativeness of the cases**	**Selection of controls**	**Definition of controls**	**Ascertainment of exposure**	**Same method of ascertainment for cases and controls**	**Non-response rate**		
Mediratta 2020	★	★	-	★	★	★	★	6	Good
Money 2024	★	★	-	★	★	★	★	6	Good

eTable 4. Adaptation and interpretation of Newcastle-Ottawa Scale for Case-Control Studies

Thresholds for converting the Newcastle-Ottawa scales to Agency for Healthcare Research and Quality standards (good, fair, and poor):

Good quality: 3 or 4 stars in selection domain AND 1 or 2 stars in comparability domain* AND 2 or 3 stars in outcome/exposure domain
Fair quality: 2 stars in selection domain AND 1 or 2 stars in comparability domain* AND 2 or 3 stars in outcome/exposure domain
Poor quality: 0 or 1 star in selection domain OR 0 stars in comparability domain* OR 0 or 1 stars in outcome/exposure domain

*As we removed the comparability domain for this study, we removed this summary grading criteria

Adapted Newcastle Ottawa Quality Assessment Scale for Case-Control Studies:
Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Exposure categories.

Selection
1) Is the case definition adequate?
    a) Independent blind assessment - 1 point
    b) Record linkage (culture/lab-confirmed infection or death) - 1 point
    c) Unblinded assessment - 0 points
    d) No description - 0 points
2) Representativeness of the cases
    a) Consecutive or obviously representative series of cases - 1 point
    b) Potential for selection biases or not stated - 0 points
3) Selection of Controls
    a) Community/outpatient controls - 1 point
    b) Hospital controls - 0 points
    c) No description - 0 points
4) Definition of Controls
    a) No history of disease (endpoint) - 1 point
    b) No description of source - 0 points

Exposure
1) Ascertainment of exposure
    a) Secure medical record - 1 point
    b) Structured interview where blind to case/control status - 1 point
    c) Interview not blinded to case/control status - 0 points
    d) Written self report (parent/caregiver reported all signs in paper) - 0 points
    e) No description - 0 points
2) Same method of ascertainment for cases and controls
    a) Yes - 1 point
    b) No - 0 points
3) Non-Response rate
    a) Same rate for both groups - 1 point
    b) Non respondents described - 0 points

c) Rate different and no designation - 0 points
Questions were tailored to reflect the research question of this study: 1. We removed the Comparability section (see reasoning in NOS Cohort section above) 2. We tailored the ascertainment of exposure (Exposure Question 1 option a) as the original scale downgraded for use of medical records only. Many chart review studies are included in this review and are deemed appropriate.

eFigure 4. Detailed individual study-level risk of bias (ROB) chart: QUADAS-2

Study	Risk of bias					Applicability		
	Participant selection	Index test	Reference standard	Participant Flow	Overall	Participant selection	Index test	Reference standard
Bohnhorst 2012	+	+	+	+	Not serious	-	+	+
Bonadio 1991	+	+	+	+	Not serious	+	+	+
Brady 1993	+	+	?	?	Serious	+	+	-
Coggins 2024	+	+	+	+	Not serious	-	+	+
Das 2016	-	+	+	+	Serious	-	+	+
Diaz Alvarez 1996	+	+	+	+	Not serious	-	+	+
Guo 2023	+	+	+	+	Not serious	-	+	+
Hiew 1992	+	+	+	?	Not serious	+	+	+
Hofer 2012a	+	+	+	+	Not serious	-	+	+
Hofer 2012b	+	+	+	+	Not serious	-	+	+
Singh 2003	+	+	+	+	Not serious	-	+	+

eTable 5. Adaptation and interpretation of QUADAS-2 tool

We followed the standard QUADAS-2 signaling questions and recommended downgrading were used to grade each domain. Grading of certain domains required determining additional rules specific to the context of our research question:
• ROB of participant selection was downgraded if a case-control study design was used.

• ROB of index test was downgraded if all clinical signs reported in the paper were assessed by historical report alone. ROB of index test was also graded as high (red) if the index test was ascertained without an interview (form) and no discussion with study staff.
• ROB of the reference standard was downgraded if its assessment was not blinded to the index test assessment. For the reference standard using laboratory diagnosis, we assumed that the person interpreting the reference standard was blinded to the person assessing the index test (clinical signs) unless otherwise stated.
• For ROB of participant flow, we graded studies as low (green) if the interval of time between index test and reference standard assessments was less than four hours, unclear (yellow) if the interval of time between index test and reference standard assessments was four to 12 hours, and high (red) if the interval of time between index test and reference standard assessments was greater than 12 hours. ROB of participant flow was also graded as high (red) if the study did not include all enrolled participants in the analysis.
• For applicability of participant selection, we graded studies as having high concern if they were conducted in neonatal intensive care units (NICUs).
Overall ROB for each study was graded as follows: 'not serious' if <2/4 ROB domains were unclear or <1/4 domains were high; 'serious' if 2/4 domains were unclear or 1/4 domains were high; and 'very serious' if >2/4 domains were unclear or >1/4 domains were high.

eFigure 5. Detailed individual study-level risk of bias (ROB) chart: QUAPAS

Study	Risk of bias						Applicability			
	Participants	Index test	Outcome	Flow and timing	Analysis	Overall	Participants	Index test	Outcome	Flow and Timing
Pathak 2019	+	+	+	+	+	Not serious	+	+	+	+
Widyaningsih 2025	+	+	+	?	?	Serious	+	+	+	+

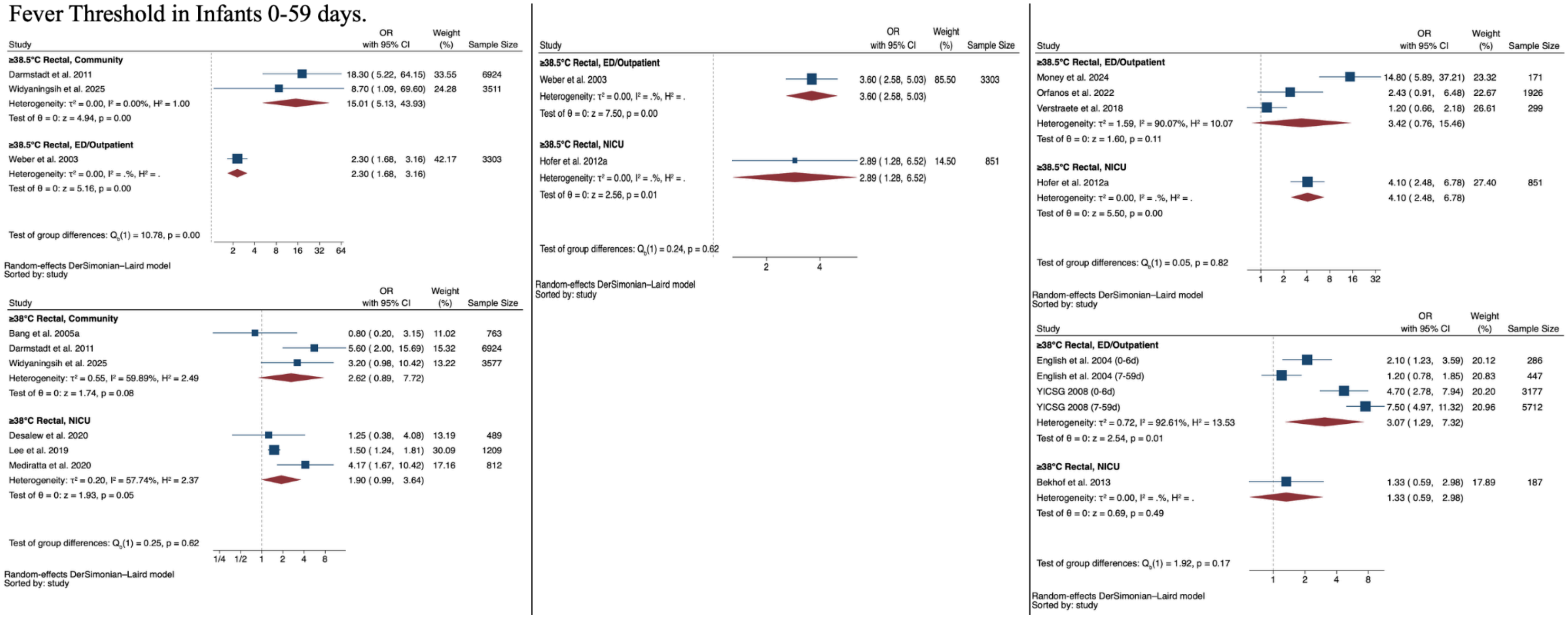
eTable 6. Adaptation and interpretation of QUAPAS tool

We followed the standard QUAPAS signaling questions and recommended downgrading were used to grade each domain. Grading of certain domains required determining additional rules specific to the context of our research question:
• ROB of participant selection was downgraded if a case-control study design was used.
• ROB of index test was downgraded if all clinical signs reported in the paper were assessed by historical report alone. ROB of index test was also graded as high (red) if the index test was ascertained without an interview (form) and no discussion with study staff.

● ROB of the outcome was downgraded if its assessment was not blinded to the index test assessment. For the reference standard of mortality, we assumed that the person interpreting the reference standard was blinded to the person assessing the index test (clinical signs) unless otherwise stated.
● ROB of flow and timing was downgraded if participants were followed up until less than the end of their defined cohort age
● For applicability across all domains, we followed the standard QUAPAS signaling questions.
Overall ROB for each study was graded as follows: ‘not serious’ if <2/5 ROB domains were unclear or <1/5 domains were high; ‘serious’ if 2/5 domains were unclear or 1/5 domains were high; and ‘very serious’ if >2/5 domains were unclear or >1/5 domains were high.

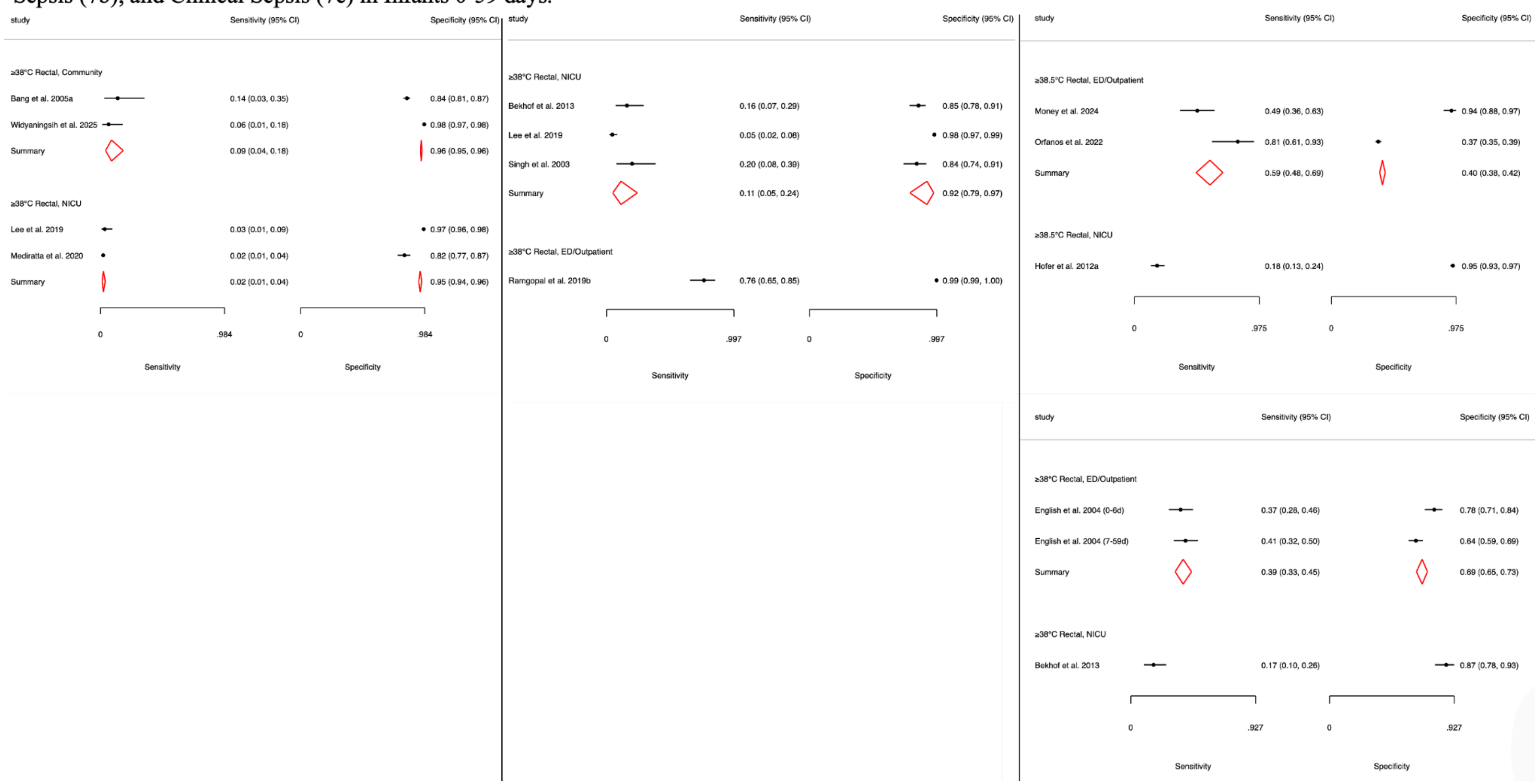
eFigure 6a-c. Subgroup Analyses by Clinical Setting: Pooled ORs for Mortality (6a), Culture-Confirmed Sepsis (6b), and Clinical Sepsis (6c) by Fever Threshold in Infants 0-59 days.

Supplementary Figure 6a-c. Subgroup Analyses by Clinical Setting: Pooled ORs for Mortality (6a), Culture-Confirmed Sepsis (6b), and Clinical Sepsis (6c) by Fever Threshold in Infants 0-59 days.



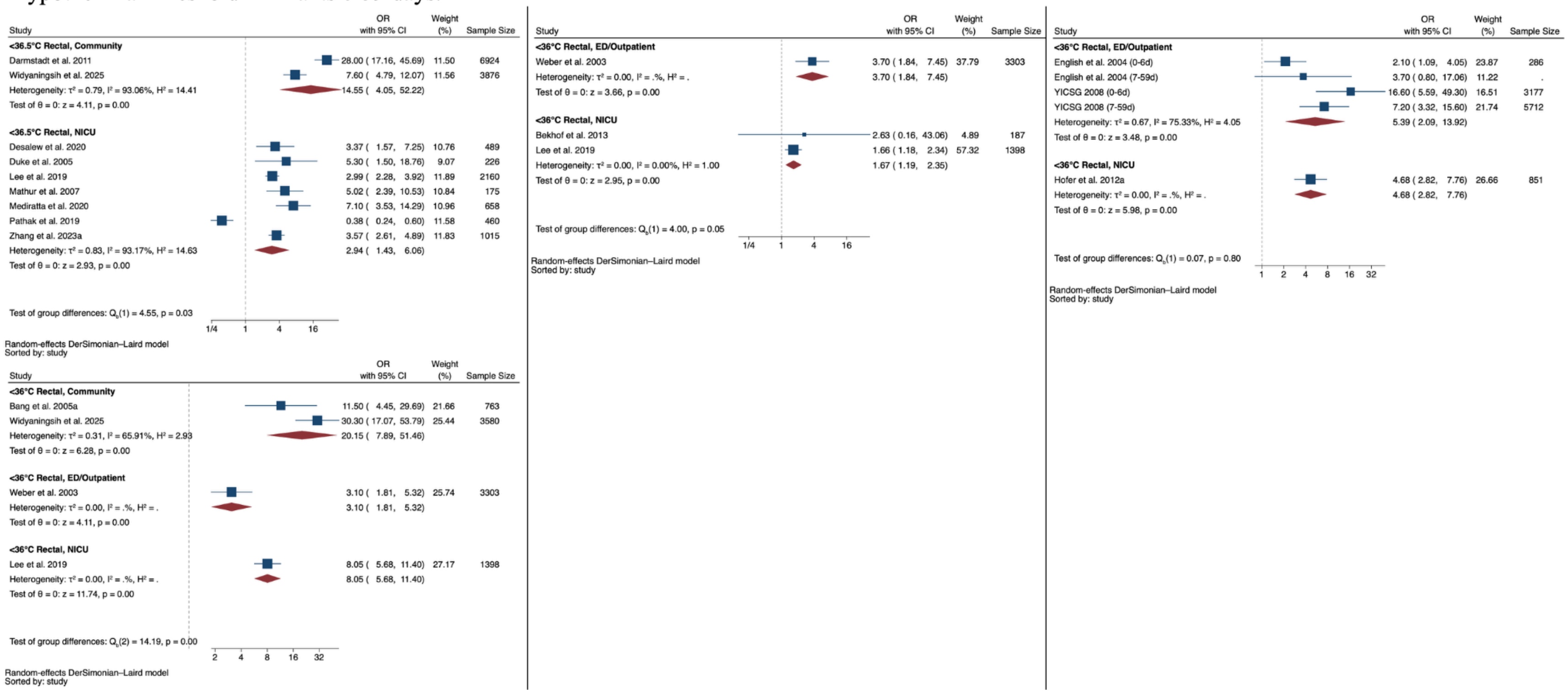
eFigure 7a-c. Subgroup Analyses by Clinical Setting: Sensitivity and Specificity of Fever Thresholds for Mortality (7a), Culture-Confirmed Sepsis (7b), and Clinical Sepsis (7c) in Infants 0-59 days.

Supplementary Figure 7a-c. Subgroup Analyses by Clinical Setting: Sensitivity and Specificity of Fever Thresholds for Mortality (7a), Culture-Confirmed Sepsis (7b), and Clinical Sepsis (7c) in Infants 0-59 days.



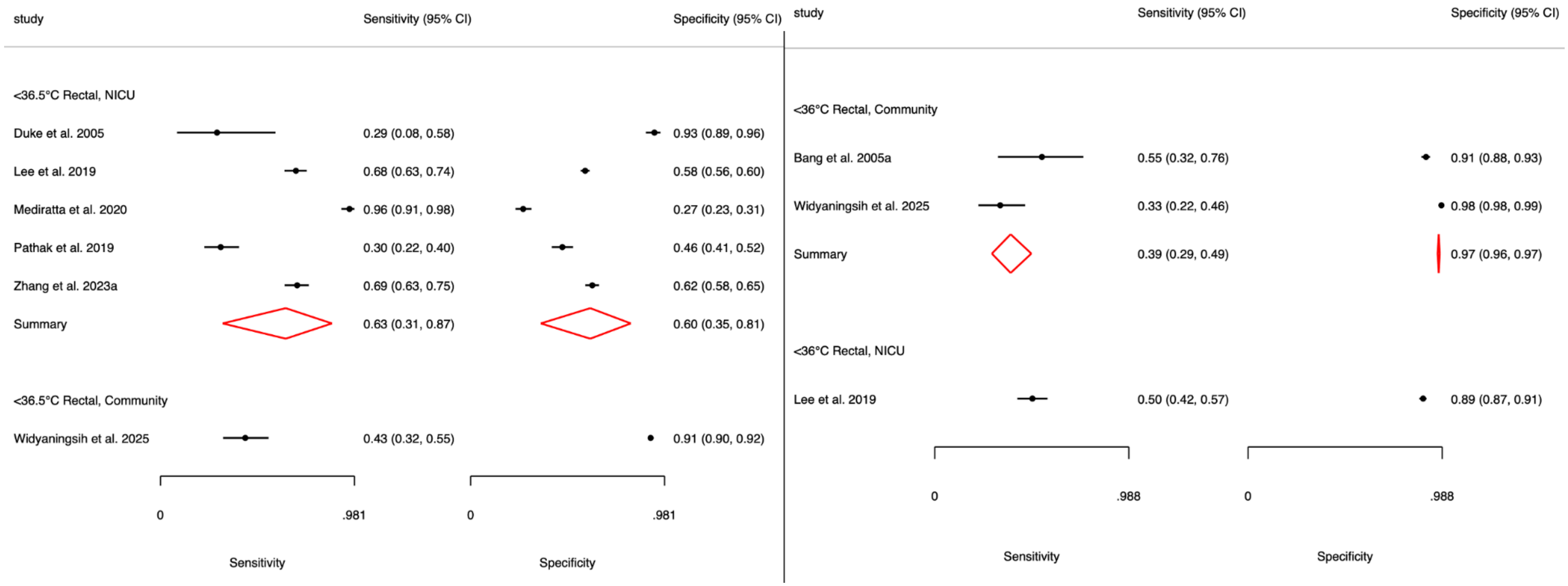
eFigure 8a-c. Subgroup Analyses by Clinical Setting: Pooled ORs for Mortality (8a), Culture-Confirmed Sepsis (8b), and Clinical Sepsis (8c) by Hypothermia Threshold in Infants 0-59 days.

Supplementary Figure 8a-c. Subgroup Analyses by Clinical Setting: Pooled ORs for Mortality (8a), Culture-Confirmed Sepsis (8b), and Clinical Sepsis (8c) by Hypothermia Threshold in Infants 0-59 days.



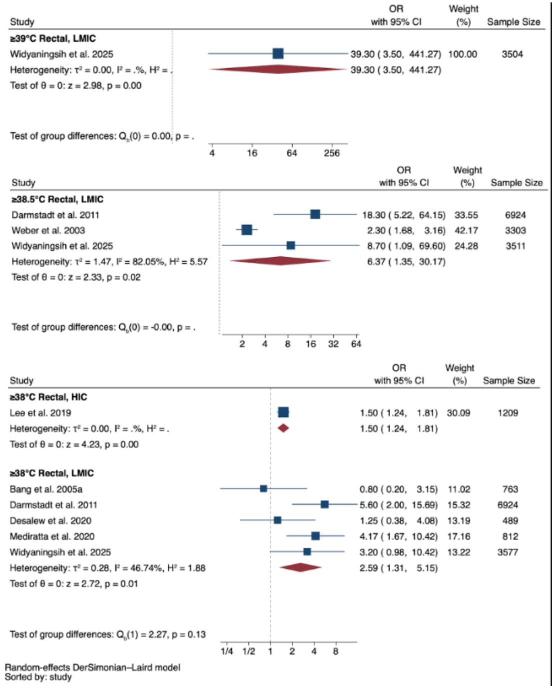
eFigure 9a-c. Subgroup Analyses by Clinical Setting: Sensitivity and Specificity of Hypothermia Thresholds for Mortality (9a), Culture-Confirmed Sepsis (9b), and Clinical Sepsis (9c) in Infants 0-59 days.

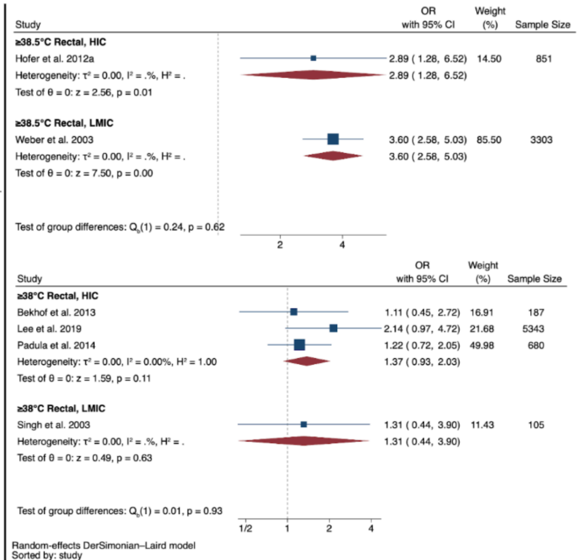
Supplementary Figure 9. Subgroup Analyses by Clinical Setting: Sensitivity and Specificity of Hypothermia Thresholds for in Infants 0-59 days.

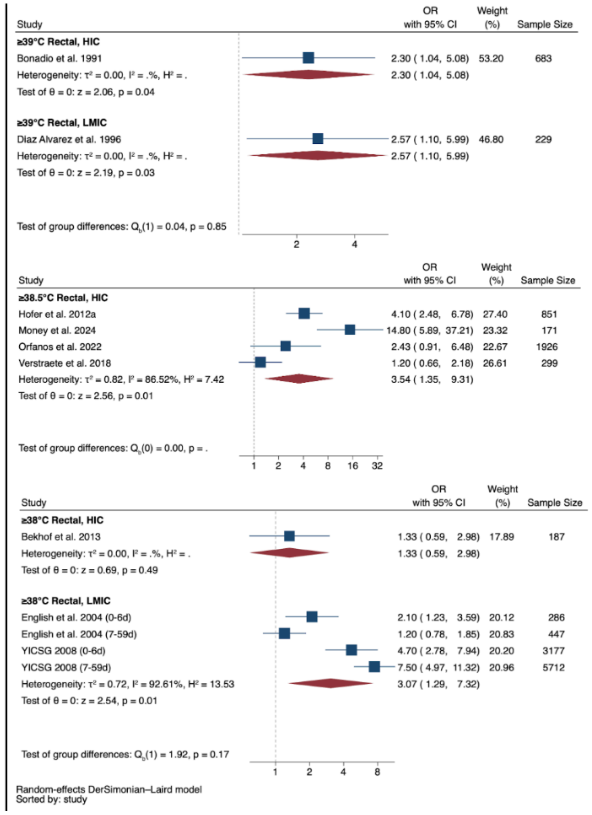


**Insufficient studies to stratify by clinical setting for culture-confirmed sepsis and clinical sepsis.

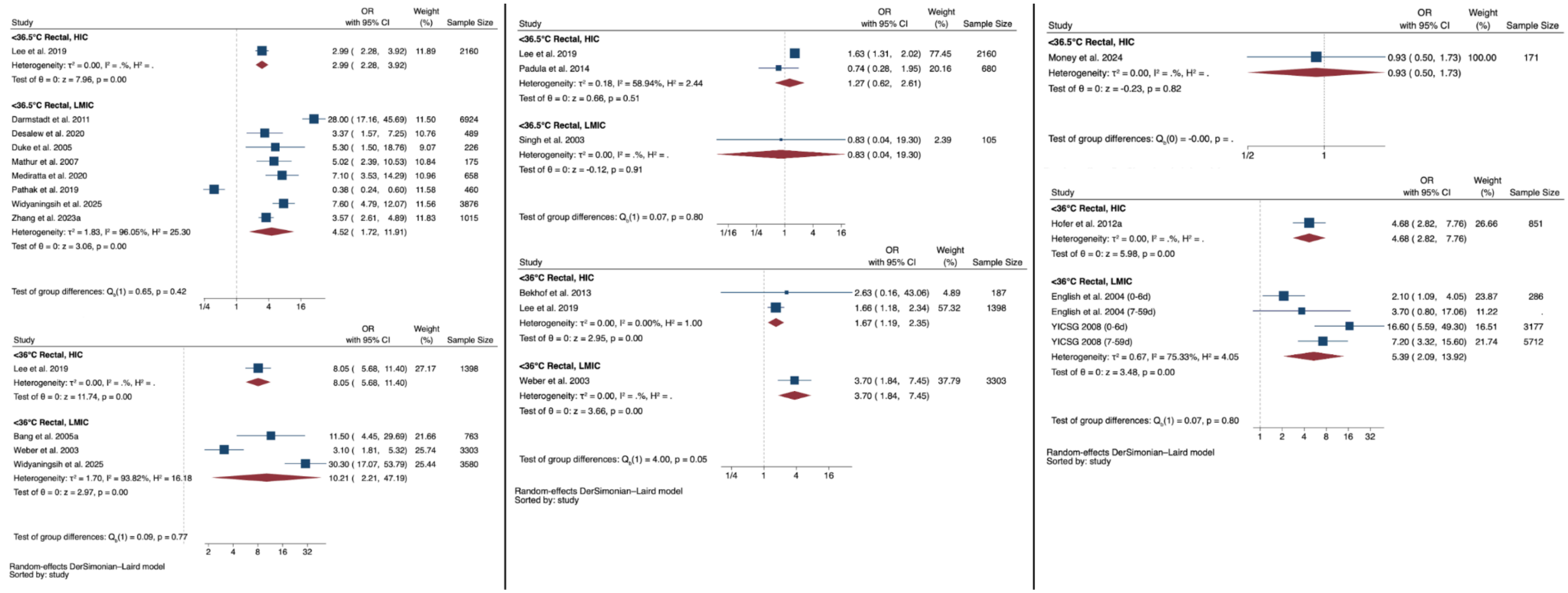
eFigure 10a-c. Subgroup Analyses by Country Income: Pooled ORs for Mortality (10a), Culture-Confirmed Sepsis (10b), and Clinical Sepsis (10c) by Fever Threshold in Infants 0-59 days.



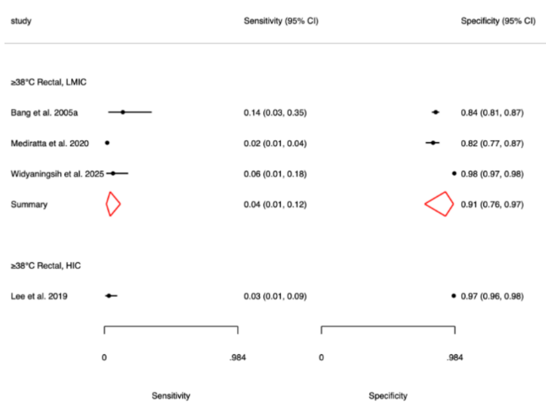




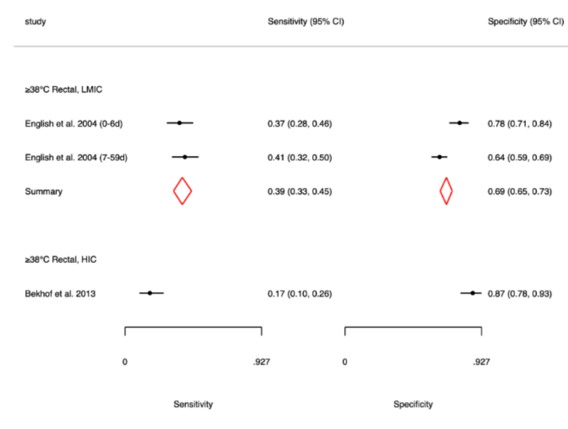
eFigure 11a-c. Subgroup Analyses by Country Income: Pooled ORs for Mortality (11a), Culture-Confirmed Sepsis (11b), and Clinical Sepsis (11c) by Hypothermia Threshold in Infants 0-59 days.



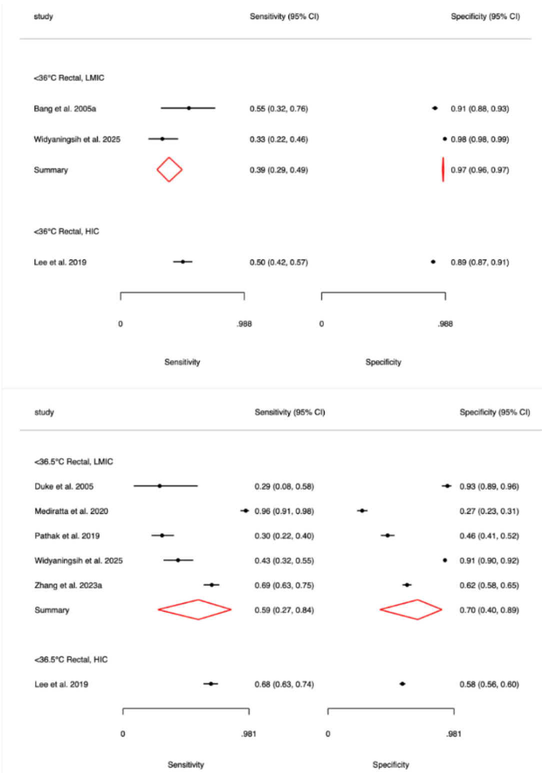
eFigure 12a-c. Subgroup Analyses by Country Income: Sensitivity and Specificity of Fever Thresholds for Mortality (12a), Culture-Confirmed Sepsis (12b), and Clinical Sepsis (12c) in Infants 0-59 days.



Insufficient studies for additional stratification.



eFigure 13a-c. Subgroup Analyses by Country Income: Sensitivity and Specificity of Hypothermia Thresholds for Mortality (12a), Culture-Confirmed Sepsis (12b), and Clinical Sepsis (12c) in Infants 0-59 days.



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